AC-06 Adult Narrow-Complex Tachycardia

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults age 16 years or older with a pulse and a narrow-complex tachycardia. Treat the patient and the cause, not the rate alone.

Narrow-complex rhythm classification and treatment

1. Assess context	Airway/ventilation, oxygen only if hypoxemic, monitor/pads, frequent BP, IV/IO, and 12-lead when it will not delay urgent care. Treat sinus-tachycardia causes.
2. Instability caused by rhythm?	Hypotension, acute altered mental status, shock, ischemic chest discomfort, or acute heart failure.
3. Unstable	Synchronized cardioversion: regular narrow 100 J; atrial fibrillation or flutter 200 J. Sedate when feasible but never delay shock; confirm synchronization before every attempt.
4. Stable regular narrow	Modified Valsalva; no carotid massage. If unsuccessful, adenosine 6 mg rapid IV/IO plus immediate flush, then 12 mg after 1-2 minutes if needed; may repeat 12 mg once.
5. Stable AF/flutter with RVR	No routine adenosine. Use rate control only when significantly symptomatic, SBP is at least 110 mmHg, and no AV-nodal-agent contraindication is present.
6A. Cardizem available	Diltiazem 0.25 mg/kg IV/IO (maximum 20 mg); if needed, 0.35 mg/kg IV/IO after 15 minutes (maximum 20 mg). An infusion of 5-10 mg/hr requires Medical Control.
6B. Cardizem unavailable - approved shortage alternative	Metoprolol 2.5-5 mg IV push over 1 minute every 10 minutes; maximum 3 doses/15 mg. Reassess BP, HR, rhythm, perfusion, respiratory status, and symptoms before every dose.

Key treatment and handoff

Item	Approved action
Do not use either AV-nodal drug	Hypotension, cardiogenic shock, acute decompensated HF/suspected systolic dysfunction, ACS, pre-excited AF/flutter, bradycardia/advanced heart block, or undifferentiated wide-complex rhythm.
No adequate response/intolerance	Contact Medical Control. Do not independently combine or sequence diltiazem and metoprolol, and do not use amiodarone as an AV-nodal substitute in this stable AF-with-RVR branch.

The metoprolol branch is a single shortage contingency, activated only while Claiborne pharmacy/operations identifies diltiazem as unavailable. It is not a second routine first-line branch.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- If pre-excited AF/flutter, an undifferentiated wide-complex rhythm, or polymorphic rhythm is suspected, do not give adenosine, diltiazem, metoprolol, digoxin, or amiodarone; open AC-07/AC-08 and contact Medical Control.

- Do not electively cardiovert stable AF/flutter of unknown duration.

- Obtain a post-treatment rhythm strip/12-lead and complete vital signs; monitor for recurrence.

Linked pathways

Pathway	Use when
AC-07	Monomorphic wide-complex rhythm
AC-08	Polymorphic wide-complex rhythm
AC-09	VF/pulseless VT if pulse lost

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.